11.1 Burns (Acute Phase)

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I am very thirsty." **Objective** • Partial- and full-thickness burns, 30% • TBSA • HR 125/min, BP 96/60 mmHg • Urine output 20 mL/hour	**ACTUAL** **Deficient fluid volume** related to capillary leak and fluid loss through burned skin as evidenced by tachycardia, hypotension and low urine output.	Within 8 hours, urine output 0.5 mL/kg/hour (adult) and vital signs stable.	• Put in two large-bore IV cannulas and start fluids by the burn formula, as prescribed. • Record urine output every hour through the catheter. • Adjust the fluids as ordered, going by the urine output. • Vital signs every hour, and follow electrolytes. • Weigh every day.	Urine output on target: **goal met**.
Subjective • "The pain is unbearable." • Gives the pain 9/10 **Objective** • Partial-thickness burns • Crying and restless	**ACTUAL** **Acute pain** related to burn injury and exposed nerve endings as evidenced by pain score 9/10 and restlessness.	Pain 4/10 or less at rest, and copes with dressing changes.	• Give IV opioids as prescribed; do not use the IM route in the acute phase. • Give analgesia 30 minutes before each dressing change. • Keep the wounds covered so less air reaches them. • Distract him and explain every step.	Pain controlled: **goal met**.
Subjective • *No specific complaint* **Objective** • Skin barrier lost • Invasive lines in place • Burn wound	**RISK** **Risk for infection** as evidenced by risk factors: loss of skin integrity and invasive devices.	No wound infection and no systemic infection.	• Strict aseptic technique for wound care. • Apply the topical antimicrobial dressings as prescribed. • Watch how the wound looks and smells, and the temperature. • Give tetanus prophylaxis as ordered.	No infection: **goal met**.